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Name: نهي عبدالفتاح محمد احمد بحري National ID: 28610012128485	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





Patient Name	: Mrs. Maryam Mohamed El-Sayed	Request No.	: 13007033
Patient No.	: 28610012128485	Request Date	: 14/02/2024
Path No.	: 625/24	Verification Date	: 25/02/2024
Sex	: Female	Financial Category	: 12345678901234567890
Age	: 37 Y	Department	: Out Patient
Referring Doc :		Registered by	: maria mohamed youss

Pathology Report

- PR Positive (score 6/8)
- Her2: (score 1+)
- Ki67: Positive in 22% of tumor cells

* Scoring system: 'ER, PR'

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: 'Her2'

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013 IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Comment: Although Her2 is Negative, such cases (Score 1+) are predictive of response to Her2-Ab drug conjugates (ADCs) according to latest 2023 ASCO-CAP guidelines.

Signature
Prof. Dr. Lubna Omar,
Dr. Taka Medhat

Verified by :
DR Menna Nabil

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Room No. (11) - First Floor - Building No. 585 - Nile Corniche - Mar El Quesr - Cairo, Egypt

Patient Name : Mrs. Mayada Hegazy
Patient No : 20610012120135
Date No : 3553/24
Sex : Female
Age : 37 Y
Referral No :
Request No : 13902480
Request Date : 22/09/2024
Verification Date : 09/10/2024
Financial Category :
Department : Out Patient
Registered by : Mrs Mayada Hegazy

Pathology Report

- * Lymph node involvement: Positive in 4/5 lymph nodes with marked extracapsular extension.
- * Resection Margins: Free deep margin.
- * Histologic response to therapy: Partial pathological response.
- * Microcalcifications: Positive.

Diagnosis:

Right breast cancer post NACT, MRM specimen.

- Bifocal invasive duct carcinoma, grade II.
- Maximal tumor diameter: 3 cm
- Actual tumor cellularity: 50%, tumor necrosis index: 5+5
- Free deep resection margin
- Free nipple and areola skin.
- Lymph nodes are metastatic in 4/5 lymph nodes with marked extracapsular extension.
- Partial pathological response to therapy.
- Stage: ypT2a, ypN2
- RCB: 3.522, RCB class III

Signature

Dr. Rana Fatmy
Dr. Toka Medhat

Verified by :

Dr. Mervat Mahmoud

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Printing By : mervat mahmoud yass

Page 2

Ministry of Health and Social Work
Ministry of Health and Social Work
Patient No: 20610012120135, Date No: 3553/24, Sex: Female, Age: 37 Y, Referral No: 13902480, Request Date: 22/09/2024, Verification Date: 09/10/2024, Financial Category: , Department: Out Patient, Registered by: Mrs Mayada Hegazy

Patient Name	Dr. Hossam Mohamed El-Mohamdy	Request No	13007033
Patient No.	28610012128485	Request Date	14/02/2024
Birth No.	625/24	Verification Date	26/02/2024
Sex	Female	Financial Category	Out Patient
Age	37 Y	Department	Out Patient
Referring Doctor		Registered by	umarwa mohamed youss

Pathology Report

Clinical Data: Right breast malignant looking solid mass lesion.

Nature of Specimen: Tru-cut biopsy, collected on 13/2/2024.

Gross Examination

- Four grayish white tissue cores up to 2 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5) & Ki67 clone (30-9).

Microscopic Examination

- Histological type: Invasive duct carcinoma.
- Grade (differentiation, nuclear pleomorphism, mitosis): (3+2+2) Grade II-III.
- Ductal carcinoma in situ: Negative.
- Perineural invasion: Negative.
- Angiolymphatic invasion: Negative.
- Microcalcifications: Negative.
- IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 80% of tumor cells (score 8/8).
- PR: Moderate nuclear reaction in 60% of tumor cells (score 6/3).
- Her2: Faint incomplete membrane reaction in >10% of tumor cells (score 1+).
- Ki67: 22% nuclear reaction of tumor cells.

Diagnosis:

Right breast malignant looking solid mass lesion, Tru-cut biopsy
Invasive duct carcinoma, Grade II-III.

IPT:

- ER: Positive (score 8/8).

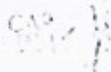
Verified by:

DR Menna Nabil

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Page 1 of 2

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Patient Name : **Dr. Mona Nabil**
Patient No. : **28610012128455**
Path No. : **625/24**
Sex : **Female**
Age : **37 Y**
Referring Doc :

Request No : **13007033**
Request Date : **14/02/2024**
Verification Date : **26/02/2024**
Financial Category : **Out Patient**
Department : **Out Patient**
Registered by : **marwa mohamed youss**

Pathology Report

- PR: Positive (score 6/8)
- Her2: (score 1+)
- Ki67: Positive in 22% of tumor cells.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

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Signature
Prof. Dr. Lubna Omar
Dr. Toka Medhat

Verified by :
DR Menna Nabil

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CAP

Room No. (10) - First Floor Building No. 987 - Nile Corniche - Madi Heliopolis - Cairo, Egypt

18/03/2024, 11:41

MDT Form - Breast Cancer



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

A-Patient ID:
1- Name: د. محمد أحمد
2-ID number: 28610012128485

Sex:

3- Medical number:
Female
Patient Age: 37 Y

63190791
Old Med:
672/2024



Family history	Phone	01144107601	BMI	REASON OF MDT	REASON OF MDT
marital status	NRS Score	GROUP	GROUP	Date of MDT	DATE OF MDT
Co morbidity	Menopausal status	Initial STAGE	Initial STAGE		18/03/2024 O

Clinical data

Breast Imaging: RT breast multicentric looking mass lesion			Mets Workup: C.T-free U/S free		
UX IOC RE COMMENT	ER 8/8 RE COMMENT	PR 6/8 RE COMMENT	Her 2 Neu 1 RE COMMENT	SISH RE COMMENT	K167 22%
Y CT4b multicentric		N 3a	M 0		

Sonomammography

PATIENT HISTORY
Echo, labs

MDT OUTCOME & PLAN

For neoadj ACx4 then Taxotere x4 then HRM

SIGNATURE

MEDICAL ONCOLOGY	RADIOLOGY	PATHOLOGY	Nutrition
SURGERY			
Psychiatric			



Date : 29/02/2024	Patient Name: Noha Abdel Fattah Mohamed
Age : 37 Year(s)	Exam : ultrasound ABDOMEN & PELVIS
	Medical ID: 672/2024

**ABDOMINO-PELVIC ULTRASOUND EXAMINATION
REVEALED:**

Liver: -

- Average in size (about 14 cm at the right MCL), smooth outline, homogenous echopattern. No intra hepatic biliary radicle dilatation.
- Portal vein is not dilated and patent.

Gall Bladder: -

- Average size, normal wall thickness, no stones.
- CBD is not dilated.

Spleen: -

- Average size (About 9 cm), displaying homogenous echopattern.

Kidneys: -

- Are of normal size and site, normal echogenicity, no stone or backpressure changes, no solid or cystic lesion.

- Para-aortic and pancreatic area: no gross lesion

URINARY BLADDER: Normal filling. Normal wall thickness, no stones or diverticulae or solid lesion detected.

Trans-vaginal ultrasound scanning revealed: -

Uterus: - anti-verted, average sized uterus. Endometrial thickness measures about: 6.

Ovaries: - both are average in size with no dominant follicle could be detected.

No free fluid in the Douglas pouch.

Opinion:

Unremarkable study

Best Regards,
Dr. Dina Gaber FEBR,

Not Approved